

# Artifact 4 — Market Segmentation Deep Dive

Market Intelligence · Stage 2

DOCUMENT DETAILS

|         |                                                                                                                                          |            |                                              |
|---------|------------------------------------------------------------------------------------------------------------------------------------------|------------|----------------------------------------------|
| Project | Home-Care-AI                                                                                                                             | Stage      | Discovery → Stage 2 (Market Intelligence)    |
| Date    | 2026-03-26                                                                                                                               | Feeds into | Artifact 5 — Opportunity Solution Tree (OST) |
| Input   | Artifact 2a (HCN), 2b (FC), 2c (CC), 2d (SR) + Artifact 3 (Competitive Gap Analysis, amended)                                            |            |                                              |
| Amended | 2026-03-26 — Quote formatting: verbatim quotes moved from inline table cells to blockquote format for consistency with Artifacts 2 and 3 |            |                                              |

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**Project:** Home-Care-AI

**Stage:** Discovery → Stage 2 (Market Intelligence)

**Skill:** market-segmentation-deep-dive

**Date:** 2026-03-26

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**Feeds into:** Artifact 5 — Opportunity Solution Tree (OST)

## 1. Scoring Framework

### 1. Scoring Framework

Per CLAUDE.md Article III Stage 2, each segment is scored on three dimensions. All scores are 1–10. **Math is shown in full.**

| Dimension                | Definition                                                                                                                                            |
|--------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------|
| Pain Severity            | How acute, frequent, and consequential is the problem? Grounded in verbatim evidence and outcome data (adverse events, hours lost, quantified costs). |
| Willingness to Pay (WTP) | Confirmed or estimated WTP adjusted for buyer type (B2C vs B2B), purchasing authority proximity, and evidence quality.                                |
| Competitor Neglect       | How poorly do current competitors serve this segment on the 6 identified gaps? Cross-referenced against Artifact 3 friction matrix.                   |

**Segment Score = Pain Severity × WTP × Competitor Neglect**

A fourth dimension — **Right to Win (RTW)** — is scored separately and used as a strategic adjustment to the raw composite, not as a multiplier. RTW answers: does the product have a credible, differentiated, defensible path to serving this segment better than alternatives?

## 2. Segment Scoring — Full Workings

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## Segment A — Home Care Nurse (HCN)

### Segment A — Home Care Nurse (HCN)

**Participants:** Maria Santos (HCN-001), David Nguyen (HCN-002), Priya Sharma (HCN-003)

**Agency:** CareBridge, eastern region. Caseloads: 8–14 patients/week direct; 50 patients oversight (Priya).

#### Pain Severity: 9/10

| Pain                                                  | Evidence                                                                                                                                                                            | Severity |
|-------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------|
| <b>Pattern blindness → preventable adverse events</b> | Maria's Arthur (hospitalised); Priya's Margaret (hospitalised). Both preventable. All warning data was in the system.                                                               | Critical |
| <b>40% trend miss rate</b>                            | Priya estimates she catches 60% of meaningful trends in her Sunday reviews — Margaret's hospitalisation was in the 40% missed.                                                      | Critical |
| <b>Documentation burden</b>                           | Maria: 2–3 hrs nightly reconstruction. David: 10–12 hrs/week including triple-checking post mg/mL error. Priya: double-documents every visit (notebook + EMR) for legal protection. | High     |
| <b>Legal defensibility</b>                            | Priya's coroner's inquest: asked to prove insulin administration time and dose — notes only said "morning visit." Professionally indefensible despite competent care delivered.     | Critical |
| <b>Medication reconciliation</b>                      | Maria: last in chain to know about dose changes. David: mg/mL near-miss → incident report → ongoing triple-check habit (+15–20 min/day). Priya: Gerald's death.                     | Critical |
| <b>Clinical loneliness</b>                            | David described making solo clinical decisions with no peer support — visibly distressed (hands shaking when recounting).                                                           | High     |

**Score: 9/10.** Two confirmed preventable hospitalisations, one coroner's inquest, one near-miss with patient safety implications, and a quantified 40% miss rate for a team lead who represents the most diligent archetype in the cohort. Frequency is daily. Consequences range from patient harm to professional registration risk.

*"I estimate I catch sixty percent of the meaningful trends in my Sunday reviews. Sixty percent is not good enough. Margaret was in the forty percent."*

— Priya, HCN-003

*"The solicitor asked me what time I gave Gerald his insulin. I said about 8:15. He said, 'Show me.' All my notes said was 'morning visit.'"*

— Priya, HCN-003

*"In the hospital, I'd ask a senior. Here, I'm on my own, standing in someone's lounge room trying to make a clinical judgment with no second opinion available."*

— David, HCN-002

#### Willingness to Pay: 7/10

| Factor               | Detail                                                                                                                                                                                              |
|----------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Buyer type           | B2B — agency pays for tools. Nurse is champion/influencer. Agency management is the economic buyer.                                                                                                 |
| Current spend signal | Agency already pays for EMR, scheduling, coordination tools. Technology budget exists.                                                                                                              |
| Maria's signal       | Price is not the barrier — usability in field conditions is the criterion.                                                                                                                          |
| Priya's signal       | Would immediately adopt any system that surfaces longitudinal trends. Key criterion: clinical credibility.                                                                                          |
| ROI case for agency  | Prevented adverse event = avoided hospitalisation cost + avoided liability exposure. Nurse overtime reduction (15–18 hrs/week → target reduction). Legal defensibility (audit-grade documentation). |
| Adoption friction    | Agency management must approve. Nurse champion alone cannot close the purchase. Political friction possible if EMR replacement threatens existing vendor relationship.                              |

**Score: 7/10.** Purchasing authority is one step removed (agency management), but the ROI case is strong and agencies have established technology budgets. Scored below FC (direct consumer) because procurement distance adds conversion friction.

*"I don't care about price — the agency pays. What I care about is whether it works on my phone between visits without crashing."*

— Maria, HCN-001

### Competitor Neglect: 9/10

| Gap                                    | Competitor Coverage                                                                                                                                       |
|----------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------|
| Longitudinal trend detection           | ✗ All three competitors — AlayaCare: per-visit only. Heidi: single encounter. Lookout Way: real-time alerts only, no multi-week synthesis.                |
| Field-context documentation            | ✗ AlayaCare crashes on mobile (own FAQ). Heidi: clinic-designed, not kitchen-designed. Lookout Way: field form completion unsupported.                    |
| Medication input validation (mg vs mL) | ✗ No competitor has point-of-entry validation for medication unit errors.                                                                                 |
| Care plan AI recommendation            | ■ AlayaCare's Recommended Care Plan Agent exists but generates drafts from assessment data only — no contraindication check, no condition-specific logic. |
| Legal-defensibility audit log          | ■ AlayaCare documents; granularity and immutability not confirmed. Heidi: AI-generated notes; field specificity clinician-dependent.                      |

**Score: 9/10.** The specific failure modes experienced by the HCN cohort (mobile crashes, no trend engine, blank care plans, no medication validation) are unaddressed or only partially addressed across all three competitors. The gap is deep, confirmed, and recurring.

### Composite Score (Raw):

$$9 \times 7 \times 9 = 567$$

### Right to Win: 8/10

| Factor                                  | Assessment                                                                                                                                                                                                                         |
|-----------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Differentiated positioning</b>       | Voice-to-structured-record (David's explicit request), pre-visit trend briefing (Maria's exact desired outcome), and AI-surfaced pattern detection with HITL confirmation (Priya's framing) are all unaddressed by any competitor. |
| <b>Clinical credibility requirement</b> | Priya's criterion — must show real data patterns, not simplistic alerts — is achievable but demands clinical validation partnership. Non-trivial.                                                                                  |
| <b>Competitive pressure</b>             | AlayaCare is moving fast: Layla, Recommended Care Plan Agent, AI Form Assistant. The window is narrowing. First-mover advantage in field-context documentation is available now.                                                   |
| <b>Moat potential</b>                   | Each visit that generates a structured, timestamped field record deepens the longitudinal dataset. The pattern detection engine improves with data. Data moat compounds.                                                           |
| <b>Risk</b>                             | AlayaCare's Recommended Care Plan Agent directly competes with the care plan feature. Differentiation must be in the clinical field context layer AlayaCare does not yet own.                                                      |

**RTW: 8/10** — Strong differentiation in the specific gaps identified. Competitive window is open but narrowing.

*"I don't need a system that replaces clinical judgment. I need a system that does the pattern recognition I can't do at scale, and then puts the pattern in front of me so I can make the judgment call."*

— Priya, HCN-003

*"I'd kill for something that just told me: 'Here's what's changed since your last visit. Here's what to watch for today.' Five sentences. That's all I need before I walk through the door."*

— Maria, HCN-001

## Segment B — Family Caregiver (FC)

### Segment B — Family Caregiver (FC)

**Participants:** Sarah Chen (FC-001), James Okafor (FC-002), Rachel Torres (FC-003)

**Context:** Adult children, sole local caregivers, working full-time. All have experienced acute gap events.

#### Pain Severity: 9/10

| Pain                                 | Evidence                                                                                                                                                                               | Severity |
|--------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------|
| Gap events — acute, life-threatening | Sarah: mother on floor 5 hrs (hip bruise, dehydration). James: father ER visit (\$5K, 6 hrs) from double-dose. Rachel: mother wrist fracture, medical alert dead during fall.          | Critical |
| Chronic anxiety                      | Rachel described constant dread between check-ins — phone calls after 9 PM trigger acute anxiety, stated as established daily fact.                                                    | High     |
| Medication adherence unverifiable    | Lin games organiser. Emmanuel double-doses (dementia). Rachel: 9 meds, 3×/day, complex interactions.                                                                                   | Critical |
| Coordination overhead                | 8–12 hrs/week across all three. Sarah: power bill as activity proxy. Rachel: physical binder weighs more than her handbag. James: handwritten geriatrician notes from 3-month guesses. | High     |
| False alarm cycle                    | All three abandoned at least one tool due to false alarms or data-without-context anxiety.                                                                                             | High     |

**Score: 9/10.** Acute events (floor, ER, fracture) plus chronic anxiety plus unverifiable medication adherence constitutes the highest objective pain density of any segment. All three pain modes are present in all three interviews.

*"I didn't know my mum was on the floor for five hours. Five hours. I was asleep in my bed forty minutes away and she was just lying there."*

— Sarah, FC-001

*"Every time my phone rings after nine PM, my heart stops. Every single time. That's not a way to live."*

— Rachel, FC-003

*"The gap between dispensed and swallowed is where everything goes wrong."*

— Rachel, FC-003

#### Willingness to Pay: 9/10

| Factor                    | Detail                                                                                                                                              |
|---------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------|
| Buyer type                | B2C — direct consumer purchase decision. No intermediary.                                                                                           |
| Confirmed spend (current) | Sarah: \$85/mo (pendant, unused). Rachel: \$95/mo (necklace + supplies). James: not quantified but already paying for baby monitor + 2 failed apps. |

| Factor                       | Detail                                                                                                                                |
|------------------------------|---------------------------------------------------------------------------------------------------------------------------------------|
| <b>Stated future WTP</b>     | Sarah: \$150–200/mo; brother splits (combined budget \$300–400/mo). James: \$200–250/mo, unprompted. Rachel: \$180/mo.                |
| <b>Evidence quality</b>      | WTP stated in response to direct pricing probe, grounded in current spend, expressed with comparative value framing.                  |
| <b>Substitution evidence</b> | All three already spending on tools that don't work. WTP is revealed preference (current spend) + stated preference (upgrade budget). |

**Score: 9/10.** Highest confirmed WTP of any segment. Direct purchasing authority. Grounded in both current spend and explicit future willingness. James's \$5K ER visit framing anchors a compelling ROI narrative.

*"The ER visit from the double dose probably cost the healthcare system five thousand dollars. One tool that prevents that pays for itself in a single incident."*

— James, FC-002

### Competitor Neglect: 8/10

| Gap                                        | Competitor Coverage                                                                                                                                                                               |
|--------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Clinical verdict interpretation</b>     | ✗ No competitor routes monitoring signals through clinical triage before family notification. Lookout Way routes to care manager first but interpretation layer is left to coordinator bandwidth. |
| <b>False alarm reduction</b>               | ✗ All monitoring tools tried by FC cohort were abandoned due to false alarms. No competitor has solved signal-to-noise at the family layer.                                                       |
| <b>Dispensed-to-swallowed verification</b> | ✗ Market-level gap (Section 4, Artifact 3). Hardware dispensers don't close it. Care platforms don't address it.                                                                                  |
| <b>Single coordination view</b>            | ■ Lookout Way family app shows roster + chat. No medication, appointment, or clinical status integration.                                                                                         |
| <b>Battery/device status to caregiver</b>  | ✗ Rachel's dead necklace: no competitor alerts the caregiver when device charge is low. The notification should go to the person who can act, not the person who forgot.                          |

**Score: 8/10.** The core FC pain (clinical verdict vs raw data) is unaddressed by every competitor. One point removed because Lookout Way partially addresses the schedule visibility component, reducing the neglect score slightly from a pure zero.

*"A verdict. Not a graph. Someone telling me: 'Your mother's patterns are within her normal range this week. Nothing to worry about.' Or: 'Something changed. Maria is looking into it.' Not a step count. A verdict."*

— Sarah, FC-001

### Composite Score (Raw):

$$9 \times 9 \times 8 = 648$$

### Right to Win: 7/10



| Factor                            | Assessment                                                                                                                                                                                                                                 |
|-----------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Differentiated product fit</b> | "Verdict not graph" is a precise product specification. Clinical interpretation layer + nurse-first routing is differentiated and unowned by any competitor.                                                                               |
| <b>Dependency risk</b>            | FC value is downstream of HCN. The clinical verdict Sarah wants requires nurses generating structured clinical data in the system. FC can't be served without HCN first. This is a Beachhead sequencing constraint, not a product problem. |
| <b>B2C acquisition cost</b>       | Consumer acquisition is expensive. FC requires marketing funnel, paid acquisition, or referral network. B2B (agency) go-to-market is leaner.                                                                                               |
| <b>Retention risk</b>             | If the product generates false alarms or raw data without interpretation, FC will abandon it — as they have abandoned every tool before. High precision required from day one.                                                             |
| <b>Complementarity</b>            | FC is the monetisation expansion layer — once HCN is in the system generating clinical data, FC has immediate upgrade willingness.                                                                                                         |

**RTW: 7/10** — High product fit, high WTP, but execution depends on HCN being onboarded first. B2C acquisition overhead is a go-to-market friction that reduces the Right to Win score relative to B2B segments.

## Segment C — Care Coordinator (CC)

### Segment C — Care Coordinator (CC)

**Participants:** Angela Morrison (CC-001, CareBridge, 60+ clients), Tom Bradley (CC-002, HomeFirst, 25 clients)

**Context:** Both are single points of failure for their agencies. Every scheduling decision, family call, and compliance obligation flows through them.

#### Pain Severity: 8/10

| Pain                                 | Evidence                                                                                                                                                                 | Severity |
|--------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------|
| Staff absence → manual cascade       | Angela: 11 phone calls per vacancy, 45 min, ~3×/week = 2.25 hrs/week reactive phone coordination.                                                                        | High     |
| 20% cancellation rate                | Tom: ~20% of vacant visits end in full cancellation. At Tom's scale: 1–2/week. At Angela's scale: 3–4/week.                                                              | High     |
| Client churn from continuity failure | Tom: Henderson family lost — 3 different carers in 2 weeks → client complaint → agency switch. Ongoing revenue loss.                                                     | Critical |
| Institutional knowledge fragility    | Angela: critical client preferences (gender requirements, refusal triggers, dementia protocols) exist only on sticky notes and in her memory — zero business continuity. | High     |
| Compliance drift / invisible gaps    | Angela: compliance audit in 6 weeks, aware of gaps but cannot quantify them. Tom: overdue care plan reviews accumulate invisibly — managed by intuition.                 | High     |
| Single point of failure              | Both coordinators: when one fire consumes bandwidth, another goes unaddressed. Families find out visits were cancelled when nobody shows up.                             | High     |

**Score: 8/10.** Pain is high-frequency, quantified (11 calls, 45 min, 20% cancellation), and has a direct client retention consequence (Henderson family churn). Scored one point below HCN and FC because physical patient harm is not the direct consequence — the consequence is operational failure, reputational damage, and revenue loss, which is serious but one severity tier below preventable hospitalisation.

*"Last Tuesday. Jenny called in sick at six-thirty AM. She had three visits booked starting at eight. I made eleven phone calls. I found a replacement for two of the three visits. The third had to be cancelled."*

— Angela, CC-001

*"If I get hit by a bus tomorrow, half the knowledge about our clients walks out the door with me. It's on sticky notes and in my head. Actually, that scares me."*

— Angela, CC-001

*"I know there are care plans overdue for review. I can feel it. But I don't have a list, I don't have a flag, just vague anxiety."*

— Tom, CC-002

#### Willingness to Pay: 8/10

| Factor                    | Detail                                                                                                                                                                                                                          |
|---------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Buyer type                | B2B — agency-level purchase. CC is champion and primary influencer. Agency director/owner is economic buyer.                                                                                                                    |
| Proximity to buyer        | Closer to purchasing decision than HCN. CC reports directly to agency management; coordinator efficiency is a management-visible problem.                                                                                       |
| Quantifiable ROI          | 2.25 hrs/week saved × coordinator hourly rate = direct cost recovery. Prevented cancellation = preserved visit fee + preserved client relationship. Henderson churn = quantified revenue loss that agency director understands. |
| Compliance audit value    | Angela's audit in 6 weeks creates an acute, time-pressured, immediately actionable use case. This is a pilot-sale opportunity.                                                                                                  |
| AlayaCare price signal    | Agencies already pay for operational scheduling software (AlayaCare, Google Sheets, etc.). Budget category exists.                                                                                                              |
| Competitive ROI narrative | AlayaCare's Vacant Visit Agent proves agencies will pay for scheduling automation. Our differentiation (soft matching layer) commands a premium over AlayaCare's current offering.                                              |

**Score: 8/10.** B2B with strong ROI narrative and close proximity to purchasing authority. One point below FC because consumer WTP evidence is more direct (stated prices) than B2B WTP (inferred from ROI).

#### Competitor Neglect: 8/10

| Gap                                              | Competitor Coverage                                                                                                                                                                               |
|--------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Soft matching (trust/preference layer)           | ■ AlayaCare's Vacant Visit Agent partially addresses vacancy filling — confirmed to match on availability/qualifications only. The "who does this patient trust" layer is explicitly unaddressed. |
| Sticky-note knowledge capture                    | ✗ No competitor captures client preferences (gender requirements, familiarity triggers, dementia briefing needs) in structured, queryable form.                                                   |
| Live compliance gap dashboard                    | ✗ No competitor provides a proactive, prioritised list of overdue care plan reviews, credential expiries, or documentation gaps. Both Angela and Tom manage compliance by intuition.              |
| Proactive family notification on schedule change | ✗ No competitor automates notification to client and family when a visit is cancelled or changed.                                                                                                 |

**Score: 8/10.** AlayaCare's partial solution (Vacant Visit Agent) moves this segment from fully neglected toward partially served — which is why it doesn't score 9 or 10. The neglect is in the specific layer that matters most: soft preference matching and the "who does this patient trust" knowledge.

#### Composite Score (Raw):

$$8 \times 8 \times 8 = 512$$

#### Right to Win: 9/10

| Factor       | Assessment                                                                                                                                                                                                                                             |
|--------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Moat clarity | Soft-preference knowledge, once captured in the system, creates immediate switching costs. Client preference profiles are relationship data — proprietary to each agency. Every visit adds a familiarity data point. Data moat compounds from day one. |

| Factor                              | Assessment                                                                                                                                                                                                                             |
|-------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>AlayaCare's gap confirmation</b> | AlayaCare's Vacant Visit Agent is direct market validation that agencies will pay for this workflow. Their gap (no trust layer) is our differentiation. This is the ideal competitive position: market proven, differentiated upgrade. |
| <b>Compliance dashboard urgency</b> | Angela's audit in 6 weeks is a named, time-pressured, immediately actionable use case. A pilot that surfaces her overdue items before the auditor does earns deep trust and a case study.                                              |
| <b>Sales motion</b>                 | B2B agency sales is leaner than B2C consumer acquisition. One signed agency contract covers multiple nurses, coordinators, and family users — efficient expansion.                                                                     |
| <b>Henderson story</b>              | Client churn from continuity failure is a Board-level business risk narrative that resonates with agency directors without requiring clinical expertise to grasp.                                                                      |

**RTW: 9/10** — The strongest Right to Win of any segment. Competitive differentiation is precise (soft matching layer above AlayaCare), moat is structural and immediate, sales motion is efficient, and there is a named acute use case (Angela's audit) ready for a pilot today.

*"It has to know the soft stuff. Not just 'who's available' — 'who does this patient trust.'"*

— Angela, CC-001

*"We lost the Hendersons. Three different carers in two weeks. Her daughter said, 'My mother doesn't know who's walking through her door anymore.' They moved her to another agency the following week."*

— Tom, CC-002

*"If the system detected the absence, found replacements ranked by match quality, sent me one approval click, and notified everyone — nobody's father would sit in a chair waiting for someone who isn't coming."*

— Tom, CC-002

## Segment D — Senior Resident (SR)

### Segment D — Senior Resident (SR)

**Participants:** Lin Chen (SR-001), Arthur Kovacs (SR-002)

**Context:** Both widowed, living independently, formal home care + family oversight. Both strategic information managers.

#### Pain Severity: 9/10

| Pain                                 | Evidence                                                                                                                                                                              | Severity |
|--------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------|
| Fall events — serious, isolating     | Lin: 5 hrs on hallway floor (hip bruise, dehydration). Arthur: 2 bathroom falls; 35-min button response wait; thought about neighbour Frank during second fall.                       | Critical |
| Weekend isolation / medication drift | Arthur: cannot reliably tell what day it is on weekends. Only bathes on visit days (self-imposed risk avoidance). Both miss doses.                                                    | High     |
| Fear of undiscovery                  | Arthur modified his entire daily routine (phone on pillow, hallway light on all night) around the fear of dying undiscovered — anchored by neighbour Frank, found 3 days after death. | Critical |
| AI trust violation                   | Previous AI system inferred depressive indicators from Arthur's voice tone → notified son and GP before Arthur → permanent behaviour change on all subsequent check-in calls.         | Critical |
| Exclusion from own care              | Both: never seen their care plans. Arthur reported ankle swelling to nurse; GP mentioned it 2 weeks later as new information — Arthur concluded reporting was pointless and stopped.  | High     |
| Data gaming                          | Lin: games pill organiser, repositions walking shoes, cancelled GP appointment to deny evidence of decline. Arthur: performs wellness on all check-in calls since AI incident.        | Critical |

**Score: 9/10.** Objectively the highest risk profile in the system. Multiple fall events, medication confusion, isolation, and an AI trust violation with documented permanent behavioural consequences. The data gaming and wellness performance findings mean objective pain is high but *observable* pain is artificially suppressed — the residents are hiding it.

*"I'm not afraid of dying. I'm afraid of dying and nobody finding me for three days."*

— Arthur, SR-002

*"I perform wellness now. That's what I call it. I perform for the phone. They taught me to hide. If I am depressed one day, truly depressed, they won't know — because I'll sound exactly the same as every other day."*

— Arthur, SR-002

*"I move the pills around so it looks right. It's not lying — it's managing. If I tell them every little thing, they'll decide I can't manage."*

— Lin, SR-001

*"I'm not afraid of dying. I'm afraid of losing my home. Everything I do is proof I can still live here."*

— Lin, SR-001

### Willingness to Pay: 2/10

| Factor                        | Detail                                                                                                                                                                         |
|-------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Purchasing authority          | None. SR is not the buyer. Agency or family pays.                                                                                                                              |
| Direct WTP signal             | Lin: would use iPad-based care info access (zero incremental cost). Will not pay for wearables. Will not wear anything. Arthur: no stated WTP.                                 |
| Adoption history              | Lin: pendant worn 2 days (mirror moment). Arthur: tablet from son unused. Two apps rejected by James's father (SR-adjacent). Fall detection watch: 3 weeks before abandonment. |
| SR-facing features in product | All are bundled into agency/family subscription. SR does not make a purchasing decision — SR makes an <b>adoption decision</b> .                                               |
| Adoption likelihood           | High risk of abandonment or active sabotage if the product is perceived as surveillance, threatens independence, or routes data to family before patient.                      |

**Score: 2/10.** SR is not the buyer and has an established pattern of abandoning or gaming any safety tool. The WTP score here represents adoption willingness (as a proxy) rather than purchase likelihood. The score is not a criticism — it reflects the correct role of SR in the commercial model: a critical design constraint and downstream user, not a paying customer.

*"If I could ask my iPad, 'When is Maria coming?' or 'What did the doctor change last time?' — I'd use that. As long as it's for Maria's work, not Sarah's anxiety."*

— Lin, SR-001

### Competitor Neglect: 9/10

| Gap                                               | Competitor Coverage                                                                                                                     |
|---------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------|
| Dignity-preserving design                         | ✗ Not addressed by any competitor. Pendant/wearable is the universal solution, universally rejected.                                    |
| Patient-facing care plan access                   | ✗ No competitor provides patient-visible care plans, visit schedules, or clinical records in accessible language.                       |
| Data gaming as design constraint                  | ✗ No competitor acknowledges adversarial patient behaviour as a design problem. All assume cooperative users.                           |
| AI inference consent + patient-first notification | ✗ Arthur's prior AI system notified family and GP before Arthur. No competitor has published a patient-first notification architecture. |
| Performing wellness / data quality degradation    | ✗ No competitor has designed around the risk that monitored patients produce false wellness signals.                                    |

**Score: 9/10.** The SR segment is the most under-designed-for in the entire market. But high competitor neglect alone does not make a viable Beachhead — because SR lacks purchasing power and has a documented tool abandonment pattern.

### Composite Score (Raw):

$$9 \times 2 \times 9 = 162$$

### Right to Win: 6/10

| Factor                            | Assessment                                                                                                                                                                                                                                                   |
|-----------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Potential if trust is earned      | Lin explicitly stated she would use iPad-based care info access. Arthur explicitly described his ideal product — advance briefing for carers on his preferences and sensitivities. Adoption is achievable with the right trust architecture.                 |
| Risk if trust is violated         | Arthur's AI incident is a lived demonstration of the downside. One inference-without-consent → permanent wellness performance → monitoring system detects nothing. The cost of a trust violation is total and permanent.                                     |
| No competitor has proven this     | This is both an opportunity (no established player) and a risk (no validated model to reference).                                                                                                                                                            |
| SR as design layer, not Beachhead | SR insights must inform the product's ethical architecture regardless of Beachhead selection. The AI Trust Architecture constraint (Section 6, Artifact 3) applies to every capability. SR is the conscience of the product, not its commercial entry point. |

**RTW: 6/10** — High potential, but trust architecture must be built correctly from the start. SR should shape design decisions across all segments, not be a standalone Beachhead.

*"When a new care worker comes, they should already know: Arthur takes his coffee black, don't move the bathroom things, ask about the cricket. That would be good. That would be care."*

— Arthur, SR-002

## 3. Scoring Summary — All Four Segments

### 3. Scoring Summary — All Four Segments

| Segment | Pain Severity | WTP | Competitor Neglect | Raw Score (P×W×C) | Right to Win | Adjusted Rank |
|---------|---------------|-----|--------------------|-------------------|--------------|---------------|
| HCN     | 9             | 7   | 9                  | 567               | 8/10         | 2nd           |
| FC      | 9             | 9   | 8                  | 648               | 7/10         | 3rd           |
| CC      | 8             | 8   | 8                  | 512               | 9/10         | 1st           |
| SR      | 9             | 2   | 9                  | 162               | 6/10         | 4th           |

## 4. Adjusted Ranking — Why Raw Score Alone Doesn't Decide

### 4. Adjusted Ranking — Why Raw Score Alone Doesn't Decide

**FC scores highest on the raw composite (648) but drops to 3rd on adjusted rank. Why?**

FC value is structurally downstream of HCN. The clinical verdict Sarah wants requires: 1. A nurse generating structured, longitudinal clinical data in the system (HCN adoption) 2. A pattern detection engine processing that data (product capability #1) 3. A clinical triage layer interpreting the signal (HITL, HCN review) 4. A verdict delivered to the family (FC output)

If HCN is not in the system, step 1 never happens. FC gets no verdicts. FC has high WTP for a product that cannot be built without HCN as a prerequisite. **FC is the monetisation layer, not the foundation layer.**

### CC scores 3rd on the raw composite (512) but rises to 1st on adjusted rank. Why?

CC has the strongest Right to Win (9/10) for three structural reasons: 1. **Moat from day one:** Soft-preference knowledge captured in the system creates immediate switching costs. The more visits recorded, the stronger the preference graph. 2. **Competitive gap confirmed:** AlayaCare's Vacant Visit Agent proves the market, but its absence of a trust/preference layer is our specific differentiation. This is the ideal competitive position: validated demand + defensible upgrade. 3. **Sales motion efficiency:** One signed agency contract covers coordinators, nurses, families, and residents at that agency. B2B is leaner than B2C consumer acquisition. And the agency is already a paying customer for someone — we are displacing, not creating, a budget line.

### HCN scores 2nd. Why not Beachhead?

HCN has the deepest clinical pain and the strongest alignment with the product's core identity (Agentic AI EMR for senior home care). But:

- Purchasing authority sits with agency management, one step further from the product's clinical champions
- CC is the operational gateway to agency adoption — closing on CC pain opens the agency door for HCN deployment
- In practice, HCN and CC are sold together in the same agency contract. CC is the door opener; HCN is the value deepener.

**The recommended commercial path: sell on CC pain → deploy for HCN use → expand to FC → design with SR.**

## 5. Beachhead Market Decision

### 5. Beachhead Market Decision

#### BEACHHEAD MARKET: Care Coordinator (CC)

#### Justification

**Right to Win is highest (9/10).** Soft-preference knowledge capture is a data moat that activates from the first client profile entered. AlayaCare has proven the market (agencies will pay for vacancy automation). Our differentiation (soft matching + compliance dashboard) is defensible and specifically unserved.

**Sales motion is the most efficient.** One agency contract = multiple nurses + coordinator + families + residents. The agency already has a technology budget (they're paying for someone). The ROI case is quantifiable from day one (minutes saved × incidents × weeks = payback period calculable in Excel by any agency director).

**Acute pilot opportunity.** Angela's compliance audit is in 6 weeks (approx 2026-05-12). A product that surfaces her overdue items before the auditor finds them earns the agency's trust, a case study, and a reference customer.

**Client churn narrative resonates at board level.** "We lost the Hendersons" is a business risk story that agency directors and owners understand immediately. It doesn't require clinical expertise to grasp — it's a revenue and reputation story.

**Gateway to HCN deployment.** Once the CC is using the platform for scheduling, HCN is the natural next user. The operational data (visit schedules, carer assignments, client profiles) that CC enters becomes the foundation for the clinical intelligence layer (trend detection, pre-visit briefings) that HCN needs. CC and HCN share a data model.



**Boundary condition:** The Beachhead is CC at the agency level. The product must solve the CC operational problem well enough to earn the agency's trust and seat at the table. Clinical features (HCN) are activated in phase 2 within the same agency.

## 6. Secondary Segments — Logged, Not Acted On Until Beachhead Validated

### 6. Secondary Segments — Logged, Not Acted On Until Beachhead Validated

Per CLAUDE.md Article III, Discovery Hard Rules: secondary segments are logged but not acted on until Beachhead is validated.

| Segment | Entry Condition                                                                                                             | Why Not Beachhead                                                                                                                                                                             |
|---------|-----------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| HCN     | Agency signed on CC features; coordinators actively using soft matching                                                     | HCN is the clinical core; enters the same agency contract as Phase 2 capability. Clinical feature set requires CC operational data as input.                                                  |
| FC      | HCN clinical data flowing consistently; at least one agency running pattern detection successfully                          | FC value requires clinical verdict from HCN-generated data. Cannot build FC product without HCN foundation. Highest WTP — best monetisation expansion.                                        |
| SR      | Trust architecture proven in production; AI inference consent model validated; no wellness performance degradation observed | SR is a design layer and ethical constraint, not a separate Beachhead. SR-facing features (care plan access, carer briefing card) are enabled within the HCN/CC product, not sold separately. |

## 7. Segment × Gap Coverage Map

### 7. Segment × Gap Coverage Map

Which gaps are felt most acutely by each segment? This feeds directly into the OST desired outcome framing.

| Gap                                         | HCN                              | FC                                  | CC                                 | SR                                         |
|---------------------------------------------|----------------------------------|-------------------------------------|------------------------------------|--------------------------------------------|
| Gap 1 — Pattern Blindness                   | ■ Primary — clinical harm        | ■ Secondary — family anxiety        | ■ Tertiary                         | ■ Tertiary                                 |
| Gap 2 — Medication Adherence                | ■ Primary — clinical risk        | ■ Primary — family caregiver burden | ■ Tertiary                         | ■ Primary — data gaming source             |
| Gap 3 — Clinically-Interpreted Family Comms | ■ Primary — data parent friction | ■ Primary — false alarm exhaustion  | ■ Secondary — family calls into CC | ■ Tertiary                                 |
| Gap 4 — Field Documentation                 | ■ Primary — daily, every visit   | ■ Not felt                          | ■ Not felt                         | ■ Not felt                                 |
| Gap 5 — Continuity / Soft Matching          | ■ Tertiary                       | ■ Tertiary                          | ■ Primary — operational crisis     | ■ Primary — stranger at door               |
| Gap 6 — Invisible Care Plan                 | ■ Secondary — quality risk       | ■ Secondary — informed coordination | ■ Primary — compliance risk        | ■ Primary — "I'd like to be a participant" |

**Key:** ■ Primary/Secondary felt acutely | ■ Not a primary pain for this segment

## 8. OST Input — Desired Outcome Statement for Beachhead

### 8. OST Input — Desired Outcome Statement for Beachhead

Per the Beachhead selection (CC), the OST Desired Outcome node is:

*"Care coordinators confidently fill every vacant visit with the right person for that patient — one approval click, zero phone calls, no father sitting in a chair — while maintaining full institutional knowledge of every client's preferences, continuity history, and trust relationships."*

Evidence anchor: Angela (CC-001) + Tom (CC-002) expressed this with identical emotional weight across two separate agencies, different sizes, different geographies. The image of a client in a chair, dressed and ready, waiting for someone who isn't coming, was unprompted in both interviews.

#### HCN Desired Outcome (Phase 2, for OST expansion)

*"Home care nurses detect the 100% of meaningful patient deterioration patterns — not 60% — before they become emergencies, in five minutes of pre-visit briefing time, not three hours of Sunday evening scrolling."*

## 9. Handshake to Next Skills

### 9. Handshake to Next Skills

| Handshake ID   | From this artifact                         | To skill                                                                                                                       |
|----------------|--------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------|
| HS-DISC-SEG-01 | Beachhead: CC + desired outcome statement  | opportunity-solution-tree — CC desired outcome is the OST root node                                                            |
| HS-DISC-SEG-02 | HCN as Phase 2 expansion + desired outcome | opportunity-solution-tree — HCN desired outcome is a secondary branch or next OST cycle                                        |
| HS-DISC-SEG-03 | FC entry condition (requires HCN first)    | brainstorm-ideas-new — FC ideas must be scoped as downstream of HCN capability                                                 |
| HS-DISC-SEG-04 | SR as design constraint, not segment       | ethics-trust-mapping — SR data gaming, AI trust violation, dignity constraints must shape Green/Yellow/Red zone classification |
| HS-DISC-SEG-05 | Full segment × gap coverage map            | opportunity-solution-tree — gap prioritisation within each opportunity node                                                    |
| HS-DISC-SEG-06 | Angela's audit date (approx 2026-05-12)    | brainstorm-experiments-new — compliance dashboard pilot is the first pretotype candidate                                       |

Artifact 4 complete. Beachhead: Care Coordinator (CC). One primary segment enters the OST. Secondary segments logged. Next step: opportunity-solution-tree — build the 4-level OST from the CC desired outcome node, using all 6 gaps and the segment × gap coverage map.